

Limye' Foundation Comprehensive Service Blueprint: Version 2.2 - Fully Integrated Plan

Executive Summary

Limye' Foundation (formerly Porch Light) is a non-profit organization positioned to transform the delivery of essential support services across Alabama. This comprehensive blueprint outlines a multi-faceted approach to securing sustainable funding through government contracts, grants, and strategic partnerships while delivering high-impact "prep services" that prepare individuals and communities for improved life outcomes.

This updated blueprint details the integration of a new, high-impact focus on the state's veteran population, building upon our established framework of serving seniors, at-risk youth, and families in crisis. The core of this expansion is "**Operation Home Base**," a synergistic initiative designed to address the most critical Social Determinants of Health (SDOH) for veterans through an innovative integration of stable housing, medically tailored nutrition, and essential transportation.

This initiative is anchored by strategic partnerships that create a sustainable and scalable service ecosystem. The "**20/59 Ventures Housing Initiative**" will be actualized through a formal partnership with a proven local developer, such as Duckworth-Morris Real Estate, to provide a dedicated inventory of safe, affordable housing units for veteran clients.¹ This stable housing creates a foundational client base for our other services. Our nutritional and transportation services will be financially sustained through a contractual relationship with the U.S. Department of Veterans Affairs (VA), primarily via its Community Care Network (CCN) administrator for Alabama, Optum.³ Furthermore, collaboration with established Veteran Service Organizations (VSOs) will enable us to access specialized funding like the VA's Highly Rural Transportation Grants (HRTG) to extend our reach into underserved communities.⁴

This veteran-focused initiative represents more than a mission expansion; it introduces a new, diversified revenue stream through VA reimbursements and a clear structure for for-profit subsidiaries, significantly strengthening the foundation's long-term financial health and sustainability. By integrating these core services, Operation Home Base will create a powerful, replicable blueprint for veteran support that reduces homelessness, improves health outcomes, and illuminates clear pathways to self-sufficiency for those who have served our nation.

Section 1: Organization Overview and Mission

Foundation Identity

- **Legal Name:** Limye' Foundation
- **Previous Name:** Porch Light
- **Tax Status:** 501(c)(3) Non-Profit Organization
- **Primary Location:** Tuscaloosa, Alabama
- **Service Area:** Statewide Alabama with focus on West Alabama region

Mission Statement (Revised)

Limye' Foundation empowers vulnerable populations, **including our nation's veterans**, through comprehensive "prep services" that bridge critical gaps in healthcare access, nutrition, **housing**, and community support, creating pathways to self-sufficiency and improved quality of life.

Vision Statement (Revised)

To be the trusted catalyst for transformative community support, connecting

Alabama's most vulnerable citizens, **with a dedicated focus on veterans**, to life-changing healthcare, nutrition, and comprehensive wrap-around services.

Core Values

- **Human Dignity:** Every person deserves respect and quality care.
- **Cultural Humility:** Honoring diverse backgrounds and experiences.
- **Innovation:** Pioneering solutions to complex social challenges.
- **Collaboration:** Building bridges between services and communities.
- **Accountability:** Measuring and maximizing our impact.
- **Service & Sacrifice:** Honoring the unique contributions and needs of veterans and their families.

Core Service Categories

1. **Healthcare Navigation & Social Determinants of Health (SDOH):** Community Health Worker (CHW) programs, Medicare SDOH risk assessments, care coordination, health literacy, and chronic disease management support.
2. **Nutritional Services:** Medically Tailored Meals (MTM), senior meal programs, institutional food services, and nutrition education.
3. **Transportation Services:** Non-Emergency Medical Transportation (NEMT), wheelchair-accessible transport, general community transportation, and meal delivery logistics.
4. **Educational & Youth Services:** Youth diversion programs, after-school enrichment, life skills training, and career readiness.
5. **Housing & Community Support:** This category is elevated in prominence with the new veteran initiative, encompassing transitional housing assistance, rapid rehousing, homelessness prevention, family resource centers, and benefits enrollment assistance.
6. **Institutional Services:** Correctional facility support, group home services, and school system partnerships, with **Military and Veteran Support Services** now established as a primary service line.

Section 2: Comprehensive Market Analysis

Alabama Procurement Landscape Overview

Alabama's procurement system operates through multiple channels:

State-Level Systems:

- Alabama Buys (Alabamabuy.gov): Primary e-procurement system
- STAARS Vendor Self Service (VSS): Legacy system during transition
- Agency-Specific Portals: Individual department procurement sites

Local Systems:

- BidNet Direct: Centralized platform for state/local opportunities
- County-Specific Portals: 67 individual county systems
- Municipal Procurement Sites: City-level opportunities

Detailed Contract Opportunities Analysis

A. Inmate Meal Services

1. Alabama Department of Corrections (ADOC)

○ **Statewide Opportunities:**

- Current RFP Example: RFP 2025-02 for Comprehensive Nutritional Food Services
- Issue Date: March 17, 2025
- Due Date: May 23, 2025
- Implementation: October 1, 2025

○ **Scope:** Complete nutritional food service management for all state facilities

○ **Requirements:**

- Minimum 100 meals/day experience
- \$250,000 bid bond

- \$12,000,000 performance bond
 - Compliance with USDA dietary guidelines
 - Alabama Board of Examiners for Dietetic/Nutritionist Practice Act compliance
 - **Subcontracting Strategy:**
 - ADOC allows subcontracting with prior approval
 - Prime contractors may need specialized services: Religious/cultural meal preparation, Therapeutic diet management, Local food sourcing, Staff training programs
2. **County and City Jail Opportunities**
- **Mobile County Metro Jail:** Comprehensive correctional food services, 3 meals/day, 365 days/year, Special dietary accommodations, 3-year terms with renewal options, On-site preparation requirements, 3-day emergency food supply maintenance
 - **City of Hoover Jail (Bid #24-012):** Frozen, individually packaged meals, Sealed bid process, Public bid openings
 - **Jefferson County Sheriff's Office:** Three meals daily service, Previous RFP due November 6, 2017, Ongoing service needs
 - **Madison County:** Current provider: Summit Food Service, "INMATE FOOD and PREP ITEMS" bids, County Commission approval required
 - **Special Consideration - Sheriff's Food Bill Law:** Depression-era law allows sheriffs to retain unspent food funds, Creates unique procurement dynamics, Requires direct engagement with county commissions, May limit competitive bidding in some counties

B. Senior Meal Services

1. **Alabama Department of Senior Services (ADSS)**
 - **Elderly Nutrition Program (ENP):** Serves 60+ seniors, spouses, and disabled residents, Over 335 senior center locations, Two service models: Congregate meals (hot lunches, M-F) and Home-delivered meals (daily hot or weekly frozen), Current Contractor: TRIO Community Meals, 5-year contract (July 1, 2023) with 5 one-year extension options
 - **Nutritional Requirements:** Compliance with latest U.S. dietary guidelines, Minimum 1/3 daily dietary reference intakes, Registered Dietitian oversight, Federal and state funding with participant donations
2. **Area Agencies on Aging (AAAs)**

- Alabama has 13 AAAs coordinating county-level services.
 - **West Alabama Regional Commission AAA:** Counties: Bibb, Fayette, Greene, Hale, Lamar, Pickens, Tuscaloosa. Services: Meal coordination, senior center operations, in-home assistance. Actively recruiting contractors for senior activity centers.
 - **Aging South Alabama AAA:** Counties: Mobile, Baldwin, Escambia. Direct service provision and contracted services. 2024 Title III Funds application available.
 - **Southeast Alabama Regional Commission on Aging (SARCOA):** "Contractor Connection" portal, Medicaid Waiver Programs, Title III Programs, Continuous need for service providers.
 - **AAA Service Opportunities:** Meal distribution management, Senior center operations, Transportation to meal sites, Nutrition education programs, In-home support services, Caregiver support programs.
3. **SNAP Restaurant Meals Program (RMP)**
- State option for SNAP benefits, Eligible populations: 60+ seniors, disabled, homeless, Restaurant partnership opportunities, Advocacy and facilitation roles.

C. Youth Services Contracts

1. **Alabama Department of Youth Services (DYS)**
 - **Diversion Grants Program:** Nearly 50 community programs, 56 counties served, Thousands of youth annually, Purpose: Alternatives to state custody, 2025 Fiscal Year opportunities available.
 - **Service Requirements:** Needs assessments, Treatment planning, Clinical therapy, In-home services, Parenting education, Transportation, Community service supervision, Family counseling, Anger management, Follow-up care, Social skills development, Educational support.
2. **Alabama Department of Human Resources (DHR)**
 - **Resource Management Division Contracts:** State and county level negotiations, Multiple provider types accepted, Faith-based organizations eligible.
 - **Contract Types:** Cost Reimbursement (Adoption, Family Services Centers) and Fixed Rate (Case Aide, Day Care, Residential Services).
 - **Current RFP Opportunities:** Intensive In-Home Services (IIHS), Family Service Centers (multiple counties including Tuscaloosa), RISE Services

(Jefferson County), Shelby County Wraparound Services, SNAP Employment & Training (A-RESET) (multiple counties including Tuscaloosa), Independent Living Programs.

D. Educational Support Services

1. Alabama State Department of Education (ALSDE)

- **Federal Programs:** Title I, Part D (Neglected/Delinquent), Title II, Part A (Teacher Quality), Title III, Part A (English Learners), Title IV, Part B (21st Century Community Learning Centers), McKinney-Vento Program (Homeless Students).
- **Current RFPs:** Career and Technical Education Professional Development, Educational resource platforms, Culinary Specialist for Farm to School.

2. CHOOSE Act Alabama

- **Education Savings Accounts (ESAs):** Refundable income tax credits, K-12 student support, "Educational therapies" included.
- **Education Service Provider (ESP) Opportunities:** Year-round application process, Direct family payment model for tutoring, specialized support.

3. Local School Districts

- **Alabama School of Mathematics and Science:** Daily food service RFPs.
- **Birmingham Board of Education:** Mental Health Services contracts.

E. Medical and Mental Health Services

1. Alabama Department of Mental Health (ADMH)

- **Professional Services Contracts:** Competitive proposal process, Current opportunities: Interpreter & CART Services, ACDD Public Awareness.
- **ACDD Innovative Ideas RFP (FY2025-02):** Due August 5, 2025, Up to \$50,000 grant, 501(c) non-profits eligible, 25% local match required, Focus on IDD population in rural Black Belt counties.
- **Substance Abuse Block Grant (SABG):** Prevention, treatment, and recovery support contract opportunities.

2. Group Home Services

- **Service Requirements:** Home-based skills training, Transportation, Wellness

supervision, Nutritious meals.

- **Funding Sources:** ADMH Adolescent Residential Program, Medicaid waivers, State appropriations.

3. **Medical Facility Partnerships**

- **UAB Health System:** Management services contracts, Strategic collaborations.
- **Alabama Rural Health Association:** Group Purchasing through Alliant for food service, medical supplies, IT services.

F. Community Development and Infrastructure

1. **Alabama Department of Economic and Community Affairs (ADECA)**

- **Grant Programs (501(c) eligible):** 2025 State Economic and Infrastructure Development (SEID), Community Development Block Grant (CDBG), Edward Byrne Memorial Justice Assistance Grant (Byrne JAG), Delta Regional Authority Programs, Southeast Crescent Regional Commission.

G. Military and Federal Opportunities

1. **US Army Contracting Command-Redstone Arsenal:** Limited "prep services" opportunities, potential subcontracting roles.
2. **Redstone Arsenal Military and Civilians' Club (RSAMCC):** Community Assistance Grants for non-profits serving military/veteran populations.
3. **Redstone Federal Credit Union (RFCU):** Community Impact Grant Program for 501(c)(3)s in specific counties.
4. **Defense Logistics Agency (DLA):** "Market Fresh" contracts for food supplies relevant to group homes.

H. Veteran Service Opportunities

Market Size and Demographics

Alabama is home to a significant veteran population, with major Department of Veterans Affairs (VA) medical centers located in Tuscaloosa and Birmingham, which serve as critical hubs for healthcare in the region.⁶ These facilities, along with their associated community-based outpatient clinics, represent the primary points of service delivery and, therefore, the primary market for Limye' Foundation's proposed veteran-focused initiatives.

The VA as a Primary Payer and the Community Care Network (CCN)

The VA MISSION Act of 2018 fundamentally transformed how the VA provides care, significantly expanding veterans' access to services from community-based providers. This positions the VA as a major healthcare and social services contractor for organizations like Limye' Foundation. A critical component of this system is the VA's use of Third-Party Administrators (TPAs) to manage its Community Care Network (CCN).³

Alabama falls within CCN Region 3, which is exclusively administered by Optum Public Sector Solutions, Inc..³ This means that for a veteran to be referred to a community provider for a covered service, and for that provider to be paid, the provider must be credentialed and contracted with Optum. Veterans become eligible for community care based on specific criteria, such as the service not being available at a VA facility, excessive drive times (e.g., >30 minutes for primary care, >60 minutes for specialty care), or extended wait times (e.g., >20 days for primary care, >28 for specialty care).⁹ Once a veteran is deemed eligible, the VA issues a referral through the CCN to an approved in-network provider.

Consequently, for Limye' Foundation to become a billable provider for VA-referred Non-Emergency Medical Transportation (NEMT) and Medically Tailored Meals (MTM), it must successfully navigate Optum's specific credentialing and contracting process. The Foundation's primary partnership for these reimbursable services will be with Optum, making this relationship a cornerstone of the initiative's financial model and sustainability.

Specific Service Market: Non-Emergency Medical Transportation (NEMT)

The VA offers several transportation solutions, including the Beneficiary Travel program for mileage reimbursement and the Veterans Transportation Service (VTS), which uses VA-operated vans.¹⁰ However, gaps in these services create a significant market opportunity. A review of the Tuscaloosa VA Medical Center's website reveals that its Disabled American Veterans (DAV) van service, a key transport option, is currently noted as "not available at this time".¹¹

This documented service disruption presents a compelling and immediate entry point for Limye's "Vets-in-Motion" program. It provides concrete evidence of a service gap that the Foundation is uniquely positioned to fill. Limye' can approach the Tuscaloosa VA's Community Care Office (contact number: 205-554-2926) with a ready-made solution to this known problem, positioning itself as a responsive and capable partner for both scheduled NEMT and overflow capacity.¹²

Specific Service Market: Medically Tailored Meals (MTM)

The veteran population has a disproportionately high rate of chronic health conditions, with 72% suffering from at least one such condition.¹³ Recognizing this, the VA has launched a high-level strategic "Food is Medicine" initiative, supported by VHA Directives and partnerships with influential organizations like The Rockefeller Foundation and MAZON: A Jewish response to hunger.¹³ The VA is actively piloting produce prescription and MTM programs, demonstrating institutional validation of this approach.¹⁴

Limye' Foundation's existing "Plates" MTM program is perfectly aligned with this VA priority. Rather than proposing a novel concept, the Foundation can position its "Plates for Patriots" program as a turnkey implementation partner for the VA's own strategic goals. Marketing materials and partnership proposals should utilize the VA's language of "whole health" and focus on improving outcomes for diet-related chronic conditions, signaling to the VA that Limye' is a knowledgeable partner ready to help them achieve their objectives.

Specific Service Market: Highly Rural Counties

To extend services beyond the Tuscaloosa-Birmingham corridor, the VA's Highly Rural Transportation Grants (HRTG) program is the key funding mechanism. This program provides grants of up to \$50,000 per eligible county to fund transportation for veterans in areas with a population density of fewer than seven people per square mile.⁴ This is an ideal vehicle for reaching veterans in surrounding counties like Greene, Hale, and Pickens.

However, federal regulations restrict HRTG eligibility to VA-recognized Veteran Service Organizations (VSOs) and State Veterans Service Agencies.⁵ As a 501(c)(3) non-profit, Limye' Foundation cannot apply directly for these funds. The only viable strategy is to forge a formal partnership with an established VSO, such as the Alabama state chapters of the American Legion, Disabled American Veterans (DAV), or Veterans of Foreign Wars (VFW).²⁰ In this model, the VSO would serve as the prime applicant and grantee, with Limye' Foundation acting as the operational subcontractor providing the vehicles, drivers, and dispatch services—a structure permitted by the grant program. This necessitates a strategic outreach effort to secure a VSO partner, which becomes a critical dependency for the rural expansion component of this plan.

Table 2.H.1: Analysis of Veteran Service Opportunities					
Service Area	Target Population	Primary Payer/Funder	Contracting Vehicle	Key Requirements	Strategic Approach for Limye'
Urban NEMT (Tuscaloosa/B'ham)	Veterans needing rides to VA or CCN appointments	U.S. Dept. of Veterans Affairs	VA Community Care Network (CCN) via Optum	Optum credentialing, NEMTAC-level driver/vehicle compliance, HSRM	Become an in-network Optum provider to receive direct referrals and bill for

				proficiency	services. Target Tuscaloosa VA's suspended DAV service as an entry point.
Rural NEMT (Underserved Counties)	Veterans in counties like Greene, Hale, Pickens	U.S. Dept. of Veterans Affairs	Highly Rural Transportation Grant (HRTG)	Partnership with a recognized VSO, grant writing expertise, adherence to HRTG reporting	Form a subcontracting partnership with a VSO (e.g., American Legion) to be the operational arm for an HRTG application.
Medically Tailored Meals	Veterans with chronic conditions (e.g., diabetes, heart disease)	U.S. Dept. of Veterans Affairs, Grants	VA CCN via Optum, Foundation/ Corporate Grants	RD-approved menus, HACCP kitchen, Optum credentialing , outcome tracking	Position "Plates for Patriots" as an implementation partner for the VA's "Food is Medicine" initiative. Seek referrals from VA dietitians and social workers.
Supportive Housing	Veterans at risk of or experiencing homelessness	HUD (VASH Vouchers), Grants, Private Donors	Master Lease Agreement, Grant Agreements	Partnership with property developer, wraparound	Partner with Duckworth- Morris to master lease a block of

				case management , knowledge of housing subsidies	units, creating a stable client base for meal and transport services.
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Section 3: Comprehensive Service Delivery Model

A. Integrated Service Hub Framework

Central Components

1. **Universal Access Point:** Single entry for all services, comprehensive intake assessment, warm handoffs between programs, "no wrong door" approach, bilingual/culturally competent staff.
2. **Service Integration Levels:**
 - **Level 1 - Co-Location:** Shared physical space, common reception, cross-program visibility, informal coordination.
 - **Level 2 - Coordination:** Shared assessments, regular case conferencing, joint service planning, resource sharing.
 - **Level 3 - Integration:** Unified case management, blended funding, shared outcomes, team-based care.
3. **Technology Infrastructure:** Integrated client management system, real-time service coordination, outcome tracking, automated referrals, mobile accessibility.

B. Core Service Programs - Detailed Implementation

Program 1: Healthcare Navigation and SDOH Services

- **Service Components:** Community Health Worker (CHW) Services (160-hour competency-based training), Medicare SDOH Implementation (Risk Assessment G0136, CHI Services G0019/G0022).
- **Partnership Model:** Formal contracts with healthcare providers (FQHCs, clinics), "incident to" billing arrangements.
- **Operational Workflow:** Referral & Consent, SDOH Risk Assessment, EMR Documentation (Z-codes), Community Health Integration services, Internal referrals, Outcome tracking.

Program 2: Comprehensive Nutrition Services

- **Medically Tailored Meals (MTM) Program ("Plates" Initiative):** Chef-prepared, RD-approved meals for specific conditions (Diabetes, Heart Disease, etc.). Home delivery, congregate dining, and grab-and-go models. Requires commercial kitchen (\$31k-\$54k), ServSafe/HACCP compliance.
- **Reimbursement Strategies:** Medicare Advantage, Medicaid waivers, HCBS programs, private insurance, hospital partnerships.
- **Senior Nutrition Services:** Congregate and home-delivered meal programs in partnership with local AAAs.
- **Institutional Food Services:** Subcontracting for correctional facilities, after-school and summer feeding programs for schools.

Program 3: Multi-Modal Transportation Services

- **Fleet Composition (Year 1):** 1-2 Wheelchair Accessible Vehicles (WAVs), 1-2 Standard Medical Transport vehicles, 1 Cargo Van. Total estimated investment: \$120,000 - \$188,000.
- **Service Types:** NEMT (Medicaid contract pursuit), General Transportation (senior

centers, employment).

- **Operational Requirements:** Annual operating costs (\$75k-\$150k), high insurance costs (\$60k-\$120k), NEMTAC driver certification.
- **Technology Integration:** Fleet management software (\$3.1k-\$4.5k) for scheduling, tracking, and billing.

Program 4: Youth and Family Services

- **Diversion Program Components:** Comprehensive assessment, individualized service plans, core services (counseling, therapy, educational support), specialized interventions (gender-specific, LGBTQ+ affirming).
- **Family Support Services:** Intensive In-Home Services (24/7 crisis availability), Family Resource Centers (one-stop access).
- **Employment Programs:** Job readiness training, interview prep, work experience placements.

Program 5: Housing and Community Support

- **Housing Assistance Programs:** Rapid Rehousing (deposit/utility assistance), Transitional Housing (short-term placements with case management), Homelessness Prevention (rental/utility aid).
- **Community Support Services:** Benefits Enrollment (SNAP, Medicaid, etc.), Crisis Intervention (24-hour hotline, mobile response).

Program 6: Limye' Veterans Initiative - "Operation Home Base"

Program Philosophy

Operation Home Base is an integrated care model designed to stabilize the lives of

veterans by addressing three core pillars of well-being: Housing, Health (Nutrition), and Access to Care (Transportation). The program's synergy is its key innovation. The housing component provides a stable client base and living environment. The nutrition component improves health outcomes and reduces medical crises. The transportation component ensures veterans remain connected to the VA healthcare system and the broader community. This closed-loop ecosystem creates efficiencies and amplifies the impact of each individual service.

Target Population

The program will serve honorably discharged veterans and their families in the West Alabama region who are experiencing or are at risk of homelessness, are facing food insecurity, or have chronic health conditions that require consistent, reliable access to VA medical care.

Component 1: Supportive Housing Partnership ("20/59 Ventures Housing Initiative")

- **Concept:** This initiative moves beyond mere housing assistance to secure a dedicated block of safe, affordable, and high-quality rental units specifically for veteran clients. The name "20/59 Ventures" is retained to honor the founder's vision and evoke the image of the I-20/59 corridor that connects the key service hubs of Tuscaloosa and Birmingham, symbolizing a pathway to stability.²¹
- **Operational Partner:** To execute this, a partnership with a proven local real estate developer and property manager is essential. **Duckworth-Morris Real Estate** is identified as the primary target partner. Their extensive portfolio of over 4,000 managed units in Tuscaloosa and explicit experience with Section 8 housing demonstrates their capacity and familiarity with federal compliance protocols necessary for serving low-income populations.¹ Other potential partners in the Tuscaloosa area include Allied Realty and Attollo Homes.²³
- **Partnership Model:** Limye' Foundation will pursue a Master Lease Agreement with the selected partner for a set number of apartment units. The Foundation will then sublet these units to veteran clients enrolled in the Operation Home Base program. Limye' case managers will provide the intensive, wraparound support

services, while the real estate partner will be responsible for standard property maintenance, ensuring a high-quality living environment.

- **Client Flow and Sustainability:** Veterans housed through this initiative become a direct, reliable, and geographically concentrated customer base for the "Plates for Patriots" meal program and the "Vets-in-Motion" transportation service. This creates an efficient, closed-loop service model, reducing logistical costs and ensuring high utilization rates for the subsidiary programs.
- **Affordability:** The financial model will leverage existing federal housing subsidies, particularly the HUD-Veterans Affairs Supportive Housing (HUD-VASH) voucher program. Grant funding from sources like the Redstone Federal Credit Union Community Impact Grant Program and community foundations will be sought to bridge any gaps and ensure rents remain deeply affordable for veteran clients.

Component 2: Veteran-Focused Nutrition ("Plates for Patriots")

- **Service Model:** This program is a specialized track of the Foundation's existing "Plates" Medically Tailored Meals (MTM) initiative. It will feature chef-prepared meals designed by a Registered Dietitian (RD) to meet the specific nutritional needs associated with chronic conditions common among veterans, such as diabetes, hypertension, and heart disease.¹³
- **VA Partnership Pathway:** A multi-pronged approach will be used to secure VA-funded clients:
 1. **Direct Referral:** Establish formal referral pathways with VA social workers, dietitians, and primary care providers at the Tuscaloosa and Birmingham VA Medical Centers.⁷
 2. **Community Care Network (CCN):** Aggressively pursue a contract with Optum to become a credentialed MTM provider. This will allow for direct billing for eligible veterans who are referred for MTM services through the CCN.
 3. **Pilot Program:** Propose a formal pilot project to the Tuscaloosa VA, leveraging the national "Food is Medicine" initiative.¹⁴ This pilot would focus on veterans in the "20/59 Ventures" housing units, allowing for the demonstration of a holistic care model with tightly controlled variables and measurable outcomes, such as reduced hospital readmissions or improved blood glucose (A1c) levels.
- **Client Base:** The initial and primary client base will be the residents of the "20/59 Ventures" housing units. This concentration dramatically increases logistical

efficiency for meal delivery, creating a dense and predictable delivery route.

Component 3: Veteran Transportation Services ("Vets-in-Motion")

- **Service Tiers:** The transportation program will operate on three distinct but integrated tiers:
 1. **Direct-to-VA NEMT:** This is the core billable service. It involves providing individual and small-group transport for veterans from their homes (including the 20/59 units) to medical appointments at the Tuscaloosa VA, Birmingham VA, and other VA-authorized community care provider offices. This service will be billed directly to the VA through the CCN via Optum.
 2. **Rural County Access (HRTG):** This tier focuses on expansion into underserved areas. By partnering with a VSO (e.g., American Legion Department of Alabama), Limye' will co-apply for HRTG funds to serve veterans in designated highly rural counties like Greene, Hale, and Pickens.⁴ Limye' will serve as the operational arm, providing the vehicles, certified drivers, and dispatching services under a subcontract with the VSO grantee.
 3. **Scheduled Inter-City Routes (Long-Term Vision):** The ultimate goal is to establish a fixed-route, scheduled shuttle service operating multiple times per week between Tuscaloosa and the Birmingham VA Health Care System.²⁶ This service would address a major regional transportation barrier for veterans needing specialized care available only in Birmingham. This can be marketed directly to the VTS coordinators at both the Tuscaloosa and Birmingham facilities as a cost-effective solution for their patient populations.⁶
- **Becoming a VA NEMT Provider:** This is the most technically demanding part of the initiative. The pathway involves:
 - **Contracting:** Initiating contact with Optum, the Region 3 TPA, to begin the network provider application and contracting process.³
 - **Credentialing:** Successfully completing Optum's credentialing process, which will rigorously assess the Foundation's licensing, insurance coverage, and operational readiness.²⁹
 - **Compliance:** Strict and continuous adherence to all VA and Optum requirements for NEMT providers, which are detailed in the Compliance section and the program SOP.

C. Limye' Families - Lifespan Approach

Intergenerational Programming

- **Children (0-12):** Early childhood development, school readiness, after-school programs, summer camps, parent-child activities.
- **Adolescents (13-18):** College preparation, career exploration, leadership development, peer support groups, creative arts programs.
- **Young Adults (19-25):** Post-secondary support, job placement, financial literacy, housing assistance, parenting classes.
- **Adults (26-59):** Career advancement, health management, caregiver support, community leadership, volunteer opportunities.
- **Seniors (60+):** Healthy aging programs, social connections, grandparent support, legacy projects, intergenerational activities.

Section 4: Organizational Structure and Staffing

Governance Structure

- **Board of Directors (7-11 members):** Board Chair, Vice Chair, Treasurer, Secretary, 3-7 At-Large Members.
- **Board Committees:** Executive, Finance, Program, Development, Governance.
- **Advisory Council:** Community leaders, service recipients, partner organizations, subject matter experts, youth representatives.

Organizational Chart (Revised)

- **Executive Level:** Chief Executive Officer, Chief Financial Officer, Chief Operating Officer, Chief Development Officer.
- **Department Directors:** Healthcare Services Director, Nutrition Services Director, Transportation Director, Youth & Family Services Director, Community Support Director, **Director of Veteran Services.**
- **Program Management:** Program Managers, Clinical Supervisors, Quality Assurance Manager, Data & Evaluation Manager.
- **Direct Service Staff:** Community Health Workers, NEMT Drivers, Kitchen Staff, Case Managers, Youth Specialists, Family Support Workers, Benefits Counselors.
- **Support Staff:** Human Resources Manager, IT Manager, Administrative Assistants, Grant Writers, Communications Coordinator, Volunteer Coordinator.

Key New and Modified Roles

- **Director of Veteran Services:** This senior position requires a candidate with significant experience in veteran benefits systems (VA, DoD), program management, and partnership development. This individual will be the primary liaison with the VA, Optum, and VSO partners.
- **Veteran Services Case Manager:** This frontline role requires a professional trained in trauma-informed care and possessing deep knowledge of VA resources, including the process for enrolling in VA healthcare via VA Form 10-10EZ.³¹ They will be the primary point of contact for veterans in the housing program, developing service plans and connecting them to resources.
- **Transportation Manager (Expanded Role):** This existing role will have its responsibilities expanded to include ensuring full compliance with VA NEMT regulations, managing driver certifications (NEMTAC, CPR/First Aid), and overseeing the detailed reporting required for HRTG grants.
- **NEMT Drivers:** These are not standard drivers. They will require specialized training beyond a commercial license, including NEMTAC certification, Passenger Assistance Safety and Sensitivity (PASS), defensive driving, and specific training on the needs of veterans with physical and psychological trauma.²⁹
- **Grant Writer (Expanded Role):** This existing role must now possess or develop expertise in federal grant applications, specifically navigating Grants.gov for opportunities like the HRTG program and other VA-specific funding announcements.¹⁸

For-Profit Subsidiary Structure for Sustainability

To generate unrestricted revenue that supports and expands the Foundation's core non-profit mission, a for-profit subsidiary structure will be established. This creates a sustainable financial engine, reducing reliance on traditional grant funding and enhancing long-term stability.

- **Limye' Transportation LLC:** This entity will house the "Vets-in-Motion" NEMT services. While its primary contract will be with the VA via Optum, this LLC structure allows for diversification into private-pay contracts, corporate shuttle services, and event transportation.
- **Limye' Foods LLC:** This entity will operate the "Plates for Patriots" program. Beyond securing VA contracts for Medically Tailored Meals, this structure enables the pursuit of other revenue-generating activities such as catering, meal prep services for the general public, and corporate wellness programs.
- **Limye' Properties LLC:** Real estate holdings, facility rentals, property management.
- **Limye' Logistics LLC:** Delivery services, supply chain management, warehouse operations.
- **Limye' Tech Solutions LLC:** IT consulting, data management, software solutions.
- **Algoinsu Insurance Brokerage:** AI-driven insurance sales, operating independently to generate commission and subsidy program revenue.

Profits generated from all subsidiaries will flow back to the Limye' Foundation to support unfunded or underfunded aspects of the mission. This dual structure allows the Foundation to maintain its 501(c)(3) status while leveraging the efficiency and market-responsiveness of for-profit entities.

Training and Certification Requirements

- **Total Initial Training Investment:** \$18,475 - \$68,535
- **Core Training Programs:** Community Health Worker Training (160-hour, \$1,500/person), NEMT Driver Certification (NEMTAC, CPR, etc., \$500/driver), Food Service Training (ServSafe, \$300/person), Clinical Staff Training

(Trauma-informed care, \$2,000/clinician), Administrative Training (Grant management, \$1,000/person).

Strategic Partnerships

- **Healthcare Partners:** DCH Health System, Whatley Health Services, Indian Rivers Mental Health, etc.
- **Community Partners:** Tuscaloosa City/County Schools, University of Alabama, United Way of West Alabama.
- **Faith-Based:** Ministerial associations, individual congregations.
- **Government:** City of Tuscaloosa, Tuscaloosa County, state agencies.
- **Business:** Chamber of Commerce, major employers.

Section 5: Technology and Equipment Infrastructure

Core Technology Systems

- **AI-Driven Management System:** CRM platform (Salesforce/HubSpot) for client tracking, service coordination, and outcome measurement. Annual cost: \$9,000-\$36,000.
- **GPS Tracking and Fleet Management:** Verizon Connect or Samsara for real-time tracking, route optimization, and maintenance alerts. Annual cost: \$3,100-\$4,500.
- **Electronic Health Records Interface:** HIPAA-compliant system with integration capabilities.
- **Financial Management:** QuickBooks Nonprofit with grant tracking modules.

Equipment Requirements

- **Commercial Kitchen Setup:** Total cost: \$31,000-\$54,000 for industrial refrigeration, commercial ranges, prep equipment, etc.
- **Vehicle Fleet (Revised Year 1):** Total Investment: \$120,000-\$188,000.
- **Office Equipment:** Furniture, computers, phone systems. Cost: \$5,000-\$10,000+.
- **Program Equipment:** Educational materials, recreation supplies, medical equipment.

Section 6: Financial Projections and Strategy

Revenue Model Overview

A diversified mix of revenue streams will ensure financial stability:

1. **Government Contracts (40-45%)**
2. **Healthcare Reimbursement (25-30%)**
3. **Grants (15-20%)**
4. **For-Profit Subsidiaries (5-10%)**
5. **Private Support (3-5%)**

Year 1 Financial Projections (Revised)

- **Revenue Projections:** Low Estimate: \$200,000; High Estimate: \$375,000
- **Revenue Breakdown:** DHR Contract (\$75k), DYS Grant (\$50k), Healthcare Billing (\$50k), Local Grants (\$25k), Private Support (\$0-\$25k), Subsidiary Income (\$0-\$150k).
- **Operating Expenses:**

Category	Low Estimate	High Estimate
Salaries and Benefits	\$150,000	\$250,000

Vehicle Operations	\$75,000	\$125,000
Insurance	\$60,000	\$120,000
Technology and Equipment	\$25,000	\$50,000
Administrative & Lease	\$45,000	\$80,000
Total Year 1 Expenses	\$355,000	\$625,000
Net Income (Loss)	(\$155,000)	(\$250,000)

Year 3 & 5 Financial Projections

- **Year 3 Revenue Target:** \$1,500,000
- **Year 5 Revenue Target:** \$3,000,000

Initial Investment Requirements (Revised)

- **Startup Costs Summary:**

Category	Low Estimate	High Estimate
Legal and Formation	\$8,600	\$17,500
Technology Systems	\$15,000	\$25,000
Office Setup	\$10,000	\$20,000
Vehicle Fleet (Revised)	\$120,000	\$188,000
Equipment	\$50,000	\$100,000
Training	\$18,500	\$68,500

Working Capital	\$50,000	\$100,000
Total Initial Investment	\$272,100	\$519,000

Grant Funding Strategy

A matrix of local, state, and federal grant opportunities will be pursued, including the Community Foundation of West Alabama, Alabama Power Foundation, The Caring Foundation, ADECA, and various federal departments (HUD, HHS).

Financial Controls and Compliance

- **Internal Controls:** Segregation of duties, monthly reconciliations, annual external audit, fraud prevention policies.
- **Compliance:** A-133 Single Audit (if applicable), IRS Form 990 filing, grant reporting, contract deliverables.

Section 7: Implementation Timeline

Year 1: Foundation Building (Months 1-12)

- **Phase 1: Legal Structure Formation (Months 1-3):** File for incorporation and 501(c)(3) status, establish bank accounts and financial policies.
- **Phase 2: Infrastructure Development (Months 4-6):** Secure office space, select technology, begin staff recruitment and partnership discussions.
- **Phase 3: Program Launch (Months 7-9):** Hire core staff, procure vehicles and equipment, launch initial CHW program, begin service delivery.

- **Phase 4: Expansion Planning (Months 10-12):** Evaluate initial programs, submit major grant applications, develop corporate partnerships.

Year 2: Growth and Stabilization

- Expand to a second county, launch nutrition and transportation services, begin Medicaid billing, launch for-profit subsidiaries.

Year 3: Scale and Impact

- Achieve 5-county service area, reach \$1.5M revenue target, serve 2,000 clients, pursue national accreditation.

Years 4-5: Statewide Leadership

- Expand to 10+ county coverage, reach \$3M annual revenue, establish an endowment, replicate the model.

Section 8: Compliance and Regulatory Requirements

Federal, State, and Program-Specific Requirements

The Foundation will adhere to all applicable regulations including:

- **Federal:** IRS 501(c)(3) rules, HIPAA, Stark Law, Anti-kickback statute, Fair Labor Standards Act.
- **State:** Alabama charitable organization registration, state tax exemption,

professional licensing, vehicle regulations.

- **Program-Specific:** DOT regulations and ADA compliance for transportation; Health department permits and USDA guidelines for food service; Background checks and mandatory reporting for youth services.

D. VA Community Care and Transportation Compliance

Adherence to VA regulations is non-negotiable and will be a primary focus of the Director of Veteran Services and the Transportation Manager. Failure in this area could result in loss of contract and reputational damage.

- **VA CCN Provider Agreement:** The foundation must sign and meticulously adhere to a network provider agreement with Optum for CCN Region 3. This legally binding document will govern all aspects of the relationship, including referrals, authorizations, claims submission procedures, and payment schedules.³
- **HealthShare Referral Manager (HSRM):** Use of the VA's secure, web-based HSRM system is mandatory for receiving referrals and authorizations from the VA.³³ All relevant staff, including the Veteran Services Director, Case Manager, and transportation dispatch personnel, must be registered with ID.me accounts and fully trained on the HSRM platform to ensure seamless coordination of care.³³
- **NEMT Provider Requirements:** The "Vets-in-Motion" program must meet the VA's stringent requirements for transportation providers, which are more rigorous than standard NEMT.²⁹
 - **Insurance:** The Foundation must secure tiered commercial auto liability insurance that meets or exceeds VA minimums. Based on standards in other states, this is projected to be at least \$350,000 for vehicles carrying 1-6 passengers and \$1,500,000 for vans carrying 7-15 passengers.²⁹ This has a direct and significant impact on the operating budget.
 - **Vehicle Standards:** All vehicles used for veteran transport must be fully ADA-compliant, properly registered, and subject to daily pre-trip safety inspections and a documented preventative maintenance schedule.¹⁹
 - **Driver Qualifications:** All drivers must possess a valid state driver's license and pass comprehensive screening, including driving record checks, criminal background checks, health screenings (DOT physicals), and a random drug testing program. They must hold and maintain current certifications in CPR, First Aid, and Defensive Driving.²⁹
- **Highly Rural Transportation Grant (HRTG) Compliance:** If successful in

securing an HRTG subcontract, the Foundation will be subject to strict federal grant reporting requirements. This includes submitting detailed quarterly and final reports to the VSO partner and the VA, documenting trips completed, unique veterans served, total distance covered, and a full accounting of how grant funds were expended.¹⁹

- **HIPAA and Veteran Privacy:** All staff must receive enhanced training on HIPAA and the specific privacy rules protecting veteran data, which are often more stringent than standard healthcare privacy laws.³²

Insurance Requirements

- **Essential Coverage:** General Liability (\$1M/\$3M), Professional Liability (\$1M/\$3M), Auto Liability (\$1M+), Property, Workers Compensation, Directors & Officers (\$1M), Cyber Liability (\$500k), Employment Practices (\$500k).
- **Annual Insurance Budget:** \$60,000 - \$120,000.

Section 9: Marketing and Community Engagement Strategy

Brand Positioning

- **Brand Promise:** "Illuminating Pathways to Possibility"
- **Core Messages:** Comprehensive support, culturally responsive services, evidence-based approaches, community-driven solutions, measurable impact.

Target Audiences

- **Primary:** Service recipients, referral partners (healthcare systems, government agencies).
- **Secondary:** Donors, funders, corporate partners, volunteers.

- **Tertiary:** Policymakers, academic institutions, general public.

Marketing Channels

- **Digital Strategy:** Professional website, social media, email marketing, online referral portal.
- **Traditional Outreach:** Community presentations, health fairs, print materials, radio partnerships.
- **Professional Networking:** Conference presentations, coalition participation.

Community Engagement Plan

- **Stakeholder Engagement:** Quarterly community forums, advisory council meetings, client feedback sessions.
- **Cultural Competency:** Multilingual materials, cultural liaisons, inclusive programming.
- **Volunteer Program:** Structured recruitment, training, and recognition system.

Section 10: Quality Assurance and Outcome Measurement

Performance Measurement Framework

- **Operational KPIs:** Client Satisfaction (Target 90%+), On-Time Performance (Target 95%+), Driver Retention (Target 85%), Safety Incidents (<1 per 10,000 miles).
- **Clinical/Program KPIs:** Health outcome improvements, hospital readmission reduction, housing stability achievement, employment placement rate.
- **Financial KPIs:** Revenue Growth (Target 25%+ annually), Operating Margin

(Target 15%+ by Year 3), Grant Success Rate (30%+).

Data Management and Reporting

- **Client Information System:** Integrated platform for real-time data access, automated reporting, and outcome tracking.
- **Quality Improvement Process:** Monthly data reviews, quarterly improvement cycles, annual program evaluation.
- **Reporting Structure:** Daily operational dashboards, weekly management reports, monthly board reports, quarterly funder reports, and an annual impact report.

Section 11: Risk Management and Mitigation

Risk Category	Specific Risk	Mitigation Strategy
Strategic	Funding Concentration	Diversify revenue streams, build cash reserves, grow earned income.
	Regulatory Changes	Monitor policy, design flexible programs, engage in advocacy.
	Competition	Emphasize unique value proposition, focus on quality and innovation.
Operational	Workforce Challenges	Offer competitive compensation, build positive culture, invest in training.
	Technology Failures	Implement backup systems, ensure cybersecurity, provide staff training.

	Service Quality	Set clear standards, provide regular supervision, utilize client feedback.
Financial	Cash Flow Delays	Secure a line of credit, establish strong collections process, maintain reserves.
	Cost Overruns	Implement monthly monitoring, strict approval processes, contingency planning.
Reputational	Service Failures / Data Breaches	Maintain robust QA, have a rapid incident response plan, ensure transparent communication.

Section 12: Procurement Portal Registration Guide

A systematic approach will be taken to register with all necessary procurement portals.

- **State-Level:** Alabama Secretary of State, Attorney General (Charitable Org), E-Verify, Alabama Buys, STAARS VSS.
- **Multi-Jurisdictional:** BidNet Direct.
- **County-Specific:** Jefferson, Mobile, Madison, Tuscaloosa, Montgomery.
- **Federal:** SAM.gov, Grants.gov.

Section 13: Strategic Recommendations

- **Immediate Priorities (0-3 Months):** Complete all core legal and procurement registrations. Target initial contracts with DHR, DYS, and ADMH. Build key relationships with healthcare providers and county commissioners.
- **Short-Term Goals (3-6 Months):** Launch operations, hire essential staff, secure

a line of credit, and begin service delivery.

- **Medium-Term Objectives (6-12 Months):** Expand to a second county, achieve \$375,000 in revenue, and launch the first for-profit subsidiary.
- **Long-Term Vision (Years 2-5):** Achieve regional leadership with 10+ county coverage, \$3M+ in annual revenue, and establish a sustainable model with over 60% earned revenue.

Section 14: Standard Operating Procedures (SOP) for Limye' Veterans Initiative

14.1 Recruitment and Intake Strategy SOP

Outreach Channels

1. **Primary Outreach (Direct VA Partnership):** Establish formal MOUs and referral protocols with the Tuscaloosa VA Medical Center ⁷ and the Birmingham VA Health Care System,²⁶ focusing on Social Work, Homeless Veteran Coordinators, and Community Care offices.¹²
2. **Secondary Outreach (VSO Partnership):** Actively engage with local VSO posts (American Legion, VFW, DAV) to present the program and build trust.⁸
3. **Tertiary Outreach (Community Engagement):** Build relationships with the Birmingham Vet Center ³⁷ and the University of Alabama's Campus Veterans Association ³⁸ to reach a broader veteran population.

Intake Process

1. **Initial Contact & Screening:** Confirm veteran status, service area, and expressed need.

2. **Benefits Verification:** Assist with VA healthcare enrollment (VA Form 10-10EZ) if needed and determine eligibility for community care.⁹
3. **Comprehensive Assessment:** Use a standardized tool to identify needs across housing, food, transport, health, and employment domains.
4. **Individualized Service Planning (ISP):** Collaboratively develop a plan outlining engagement with Limye's integrated services.
5. **Program Enrollment:** Sign all necessary consents and releases of information.

14.2 Supportive Housing Program ("20/59 Ventures") SOP

1. **Client Eligibility:** Must be enrolled in Operation Home Base with a signed ISP. Priority for homeless or at-risk veterans.
2. **Placement Process:** Case Manager assists with sublease application and subsidy paperwork (e.g., HUD-VASH), coordinating move-in with the property partner.
3. **Ongoing Case Management:** Regular in-person check-ins to ensure connection to services and mediate with property management to prevent eviction.
4. **Rent and Subsidy Management:** Finance department manages collection of tenant rent portion and receipt of subsidy payments.

14.3 NEMT Services ("Vets-in-Motion") SOP

1. **Booking a Ride (CCN Referrals):** Receive authorization via HSRM portal.³³ Dispatcher verifies authorization, books the ride in fleet software, and assigns a driver.
2. **Operational Efficiency Strategies:**
 - **Coordinated High-Volume Transport:** Work with VA schedulers to establish dedicated "Limye' Days" for specific geographic areas, allowing for efficient use of multi-passenger vans.
 - **Targeting Overflow and Service Gaps:** Serve as the primary provider for overflow demand and fill known service gaps, such as the currently suspended DAV van service at the Tuscaloosa VA.¹¹
3. **Trip Execution:** Drivers perform pre-trip safety checks, provide curb-to-curb assistance in compliance with ADA standards, and confirm return trip procedures.

4. **Post-Trip and Billing:** Driver logs trip data in real-time. Administrative staff reconciles logs and submits electronic claims to Optum for payment.²⁷
5. **HRTG and Scheduled Route Procedures:** HRTG trips are flagged as "grant-funded" for separate reporting.³⁵ The long-term vision of a scheduled shuttle will operate on a fixed route, accepting reservations and walk-ons.

14.4 Medically Tailored Meal Program ("Plates for Patriots") SOP

1. **Enrollment:** Veteran is identified via intake or direct VA provider referral. Referral must include a qualifying diagnosis and dietary prescription.¹³ Limye's RD assigns the appropriate meal plan.
2. **Production and Delivery:** Kitchen staff prepares meals adhering to RD-approved recipes. Meals are packaged, labeled, and delivered via optimized routes, focusing on the "20/59 Ventures" housing cluster for efficiency.
3. **Billing and Reporting:** For CCN referrals, claims are submitted to Optum. For grant-funded meals, data is tracked for funder reports. The RD provides quarterly summary reports to referring VA providers.

Conclusion

The integration of the "Operation Home Base" initiative into the Limye' Foundation's comprehensive service model represents a pivotal evolution for the organization. This blueprint provides a detailed, actionable, and research-backed strategy to address the complex, interconnected needs of Alabama's veterans. By moving beyond siloed services and creating a synergistic ecosystem of housing, nutrition, and transportation, the Foundation is positioned to deliver transformative, measurable outcomes.

The plan acknowledges and provides clear pathways to navigate the significant complexities of partnering with the Department of Veterans Affairs, specifically the critical necessity of engaging with the CCN administrator, Optum, and forging strategic alliances with VSOs to access specialized funding like the HRTG. The detailed Standard Operating Procedures provide the granular, on-the-ground

guidance necessary for successful implementation, risk mitigation, and compliance.

Successful execution of this plan will not only provide life-changing support to hundreds of veterans and their families but will also establish Limye' Foundation as a leader in innovative social service delivery. It creates a robust, financially sustainable model that leverages diverse revenue streams—from federal healthcare reimbursements via for-profit subsidiaries to strategic grant funding—to fuel its mission. This initiative will illuminate a new pathway to possibility, not just for the veterans we serve, but for the future of the Foundation itself.

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